fore, a new kind of human—a chimera, if it is blood or bone marrow we're discussing, or even a prosthetic chimera, if it's an electrode that was implanted in your brain."

As a physician, says Mukherjee, he doesn't just want to mitigate suffering, he wants to eradicate it. "But my very bright line is between disease and desire. Desire has to do with emancipation, augmentation, enrichment and enhancement, so I become more suspicious and careful." Not just

Mukherjee's book reads like a series of linked stories, or a film in which you see the same thing again and again, but from various angles



does that distinction between disease and desire colour much of *Song...*, it also sets up imminent crucibles. Mukherjee brings up infertility in his book and in his conversation with us. The question he asks is simple: If most people in the 1950s, including gynaecologists, held that humans were living with infertility since the dawn of time, why was it suddenly being thought of as a disease?

"There were others who realised that for some people—and it's very important to emphasise that this is only for *some people*—there's an aching associated with infertility. Now there are hundreds of thousands of babies that are born through IVF. And so, if you have this situation where the boundaries between what is considered disease and what is considered desire are constantly changing, and often in ways that we can't predict, we must find a way to understand them."

Hearing how Mukherjee likes to rid his prose of jargon and verbiage—"like the drying of a towel"—it sometimes becomes easy to forget the rather high stakes of his day job. "I'm a very particular kind of doctor. I'm a physician scientist. I invent medicines. I like making medicines, and so a lot of my reflection has to do with how to run a clinical trial." Immuneel Therapeutics, the Bengaluru-based company that Mukherjee started with Biocon founder Kiran Mazumdar-Shaw, is currently overseeing a Phase 2B trial that hopes to assess the cancer-fighting capacities of CAR (chimeric antigen receptor)-T cells. "It's a very proud moment," says Mukherjee. "But it is also a very tricky and dangerous trial. In some ways, it can be life-saving, but these are very, very difficult cells to work with."

Though Mukherjee appears to be in the habit of softening his optimism with reason, the hope in his voice is unadulterated when he tells us the story of a little boy with leukaemia, who after being administered CAR-T therapy, was pulled back from a fatal precipice. Speaking from New York, Mukherjee says, "My ambition ultimately is reverse tech transfer—to make things in India, which we can then move out or export as knowledge to the northern hemisphere and to the West. There are challenges ahead—making the therapy more accessible, reducing cost, toxicity and pain—but I do think Immuneel is realising that potential in a way I had not even anticipated when I first started this." ■

Shreevatsa Nevatia

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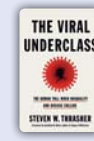
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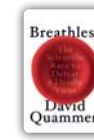
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